

1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Double-Blind Study of Adjuvant Immunotherapy with Nivolumab versus Placebo after Complete Resection of Stage IIB/C Melanoma (CA209-76K) **Brief Title:** Effectiveness Study of Nivolumab Compared to Placebo in Prevention of Recurrent Melanoma After Complete Resection of Stage IIB/C Melanoma **Short Title/Acronym:** CheckMate 76K **Protocol Number:** CA209-76K **NCT Number:** NCT04099251 **Posting ID:** 989169075546791966 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab **Phase of Development:** Phase 3 (PHASE3) **Trial Status:** Active, Not Recruiting (ACTIVE_NOT_RECRUITING) **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To determine the effectiveness of nivolumab adjuvant immunotherapy compared to placebo in adults and pediatric participants after complete resection of Stage IIB/C melanoma with no evidence of disease (NED) who are at high risk for recurrence. **Secondary Objectives:** To evaluate Distant Metastasis-Free Survival (DMFS), Duration of Treatment on Next Line Therapy Per Investigator Assessment, Progression-Free Survival Through Next-Line Therapy, Overall Survival (OS), and comprehensive safety and tolerability endpoints.

2.2 Background & Rationale Within five years after surgical resection, roughly one-third of stage IIB and one-half of stage IIC melanoma patients experience disease recurrence. To address the unmet medical need of preventing recurrence in patients with completely resected high-risk stage IIB/C melanoma, this study evaluates the efficacy of the anti-PD-1 immune checkpoint inhibitor, nivolumab, to boost the immune system against residual micrometastatic disease and provide significant recurrence-free survival benefits.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 2:1 (Nivolumab vs. Placebo)

3.2 Planned Enrollment & Duration **Targeted Enrollment:** 790 randomized patients **Number of Sites:** Multicenter, Global

Estimated Study Start: 2019 Estimated Primary Completion: 2022
(Data cut-off for interim analysis)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 12 years. 2. Participants must have been diagnosed with histologically confirmed, completely resected Stage IIB/C cutaneous melanoma with no evidence of disease. 3. Had a negative sentinel lymph node biopsy within 12 weeks prior to randomization. 4. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 5. Participant has not been previously treated for melanoma.

4.2 Exclusion Criteria 1. History of ocular or mucosal melanoma. 2. Pregnant or nursing women. 3. Participants with active known or suspected autoimmune disease or any condition requiring systemic treatment with corticosteroids (≥ 10 mg daily prednisone or equivalent). 4. Known history of allergy or hypersensitivity to study drug components. 5. Prior treatment with an anti-PD-1, anti-PD-L1, anti-PD-L2, anti-CD137, anti-CTLA-4 antibody, or agents that target IL-2 pathways, T-cell stimulators, or checkpoint pathways.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Drug Name: Nivolumab **Trade Names:** Opdivo, Opdualag **ATC Code:** L01FF01 **EMA URLs:** [Opdivo Product Information](#), [Opdualag Product Information](#) **Dosage/Frequency:** Nivolumab 480 mg every 4 weeks (or 240 mg every 2 weeks for patients ≥ 40 kg). For pediatric patients < 40 kg, the dose is 3 mg/kg every 2 weeks or 6 mg/kg every 4 weeks. **Route of Administration:** Intravenous (IV) infusion **Duration of Treatment:** Up to 1 year, or until disease recurrence, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Routine symptomatic management, hormone replacement therapies for immune-mediated endocrinopathies (e.g., hypothyroidism). **Prohibited:** Systemic immunosuppressive therapies, including corticosteroids (≥ 10 mg daily prednisone or equivalent), and concurrent administration of other investigational anti-cancer agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Up to 12 Weeks prior	Informed Consent, Sentinel Lymph Node Biopsy, Imaging (CT/MRI), Baseline safety labs
2	Every 4 Weeks	IV Infusion of Nivolumab or Placebo, AE Monitoring, Physical Exam
3	100 Days Post-Treatment	Safety Labs, Immune-

mediated AE assessment, Efficacy imaging | | **Long-Term Follow-up**
| Years 1 to 5 | Imaging every 26 weeks (Years 1-3), then every
52 weeks (Years 4-5) |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Recurrence Free Survival (RFS) is defined as the time between the date of randomization and the date of first recurrence (local, regional or distant metastasis), new primary melanoma (including melanoma in situ), or death (whatever the cause), whichever occurs first. **Censoring Rules:** For participants who remain alive and whose disease has not recurred or did not die, RFS will be censored on the date of last evaluable disease assessment. For those participants who remained alive and had no recorded post-randomization tumor assessment, RFS will be censored on the day of randomization. **Measurement Method:** Clinical assessment and scheduled radiological imaging (CT/MRI).

7.2 Secondary Outcomes

1. Distant Metastasis-Free Survival (DMFS)
2. Duration of Treatment on Next Line Therapy Per Investigator Assessment
3. Progression-Free Survival Through Next-Line Therapy
4. Overall Survival (OS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring evaluating frequency and severity per CTCAE criteria (v5.0 for reporting, v4.0 for management). Focus on immune-mediated adverse events (e.g., pneumonitis, colitis, hepatitis, endocrinopathies).

Serious Adverse Events (SAEs): Reported within 24 hours. The most common Grade 3/4 events included diarrhea and rash. **Data**

Monitoring Committee (DMC): An independent external committee monitoring safety signals and conducting pre-specified interim analyses.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population utilized for primary RFS evaluation. Safety Set utilized for all AE analyses.

Sample Size Justification: Study recruited 790 patients randomized 2:1 to provide adequate statistical power to detect a clinically meaningful HR in Recurrence-Free Survival. **Handling of**

Missing Data: Censoring applied as defined in the primary outcome measures using Kaplan-Meier methodology and Cox proportional hazards regression models for hazard ratios (HR).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring for data verification, protocol compliance, and safety review. **Audit**

Trail: Electronic Data Capture (EDC) systems utilized with full audit trails, strict adherence to eCRF locking, and formalized query resolutions.

11. Study Identifiers & Governance

Primary ID: CA209-76K **Registry Number:** NCT04099251 **Posting ID:** 989169075546791966 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 76K **Official Title:** A Phase 3, Randomized, Double-Blind Study of Adjuvant Immunotherapy With Nivolumab Versus Placebo After Complete Resection of Stage IIB/C Melanoma

12. Clinical Framework (Melanoma Specific)

Disease Area / Condition: Melanoma (Malignant Melanoma) **Semantic Type:** Neoplastic Process **Preferred UMLS Name:** Adjuvant therapy for melanoma **MeSH Code:** D008545 **HPO Code:** HP:0002861 **SNOMED ID:** 372244006 **SNOMED Hierarchy:** 1240414004|Malignant neoplasm|Malignant neoplasm (morphologic abnormality), 115223002|Nevus AND/OR melanoma|Nevus AND/OR melanoma (morphologic abnormality), 108369006|Neoplasm|Neoplasm (morphologic abnormality) **Diagnosis Threshold:** Complete surgical resection with negative margins and a negative sentinel lymph node. **Medical Methodology:** Adjuvant Checkpoint Inhibition (Anti-PD-1) **Optimization Target:** Prevention of post-surgical disease recurrence and prolonging RFS

13. Study Procedures & Methodology

Management Pathway: Adjuvant treatment initiated within 12 weeks of complete surgical resection. **Education Protocol (HCP):**

Training on identifying and managing immune-mediated adverse reactions and utilizing the provided AE management algorithms.

Education Protocol (Patient): Counseling regarding the risk of immune-mediated toxicities and immediate reporting of symptoms.

Quality Audit Mechanism: Tracking of timely radiographic assessments and adherence to dose-modification/withholding protocols for toxicities.

14. Observation & Follow-Up Schedule

Total Duration: Up to 5 years **Onsite Visit Cadence:** Every 2 to 4 weeks during the 1-year treatment phase. **Remote/Imaging Monitoring Frequency:** Radiographic disease assessments every 26 weeks for Years 1-3, and every 52 weeks thereafter for Years 4-5. **Checklist Review Interval:** Continuous AE assessments prior to every infusion dose.

15. Sign-off & Approval

Role	Name	Signature	Date					
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					